FOXO4-DRI Personal Diary

A brief record of one self-experiment

Context

At age 71, I completed three intramuscular administrations of 28 mg of FOXO4-DRI, spaced 48 hours apart. FOXO4-DRI has no established human protocol, validated human dose, or formal clinical safety record. This diary describes one uncontrolled personal experiment, not evidence of efficacy or a recommendation for use.

My survey of FOXO4-DRI research and the mechanistic extrapolation justifying my protocol is available as https://www.researcher6076.com/read/foxo4-dri .

Disclaimer: These notes should not be interpreted as definitive scientific guidance. Nothing here is medical advice for diagnosis or treatment.

Tolerability

The clearest acute effect was intense, deep injection-site pain that lasted as long as 12 hours. Splitting the final dose between both glutes made the discomfort more manageable. I also experienced temporary fatigue, itching with poor sleep, mild stomach discomfort, a headache, and a short-lived increase in chronic shoulder injury pain. I had no fever or serious adverse reaction, and all symptoms resolved within about five days after the final injection.

Immediate Outcomes

Outcomes observed and measured over the first 40 days include the following:

Dermatological changes

The most striking change was visible fading of longstanding brown age spots on my hands and scalp, together with flattening and smoothing of two raised skin lesions. By days 30 to 37, overall pigmentation on the backs of my hands appeared reduced by roughly two-thirds or more. I observed no lasting improvement in hair growth, gray hair, nocturia, or my chronic shoulder injury.

Blood work

Post treatment blood work all good showing no serious adverse results.

Interpretation

The skin changes are consistent with biological activity in at least one peripheral tissue, but this N-of-1 observation cannot demonstrate generalized senescent-cell clearance, whole-body rejuvenation, or benefit in any internal organ.

Protocol and Preparation

I had studied FOXO4-DRI for about a year before proceeding. The animal findings were encouraging, but human pharmacokinetics, dosing, safety, and efficacy were unknown. My extensive survey or the state of research on FOXO4-DRI and how I determined an experimental human dose is available at https://www.researcher6076.com/read/foxo4-dri .

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To reduce competing signals, I paused my growth-hormone secretagogue protocol about 30 days before the first injection, BPC 157 and TB 500 about 15 days before, and GHK Cu and KPV about eight days before. I also stopped heavy resistance training for a week. During the protocol I alternated thymulin and thymosin alpha 1 as immune-support measures. These choices were mechanistic and precautionary, not clinically validated.

Diary

Injection Day 1. I injected 28 mg intramuscularly into my right glute. The injection produced an intense deep ache that remained distracting for most of the day and resolved by the next morning. I felt somewhat low on energy but had a normal temperature. Sleep onset was unusually difficult that night.

Injection Day 3. The second 28 mg dose went into my left glute using less diluent. The smaller volume did not reduce the pain; the deep ache again lasted much of the day. That evening I developed a mild stomachache and noticed more pain than usual in my chronically injured shoulder.

Injection Days 4 and 5. The next day brought low energy and generalized itching, especially on my back and scalp. The itching made sleep unusually poor. For the third dose, I divided approximately 28 mg between both glutes. Pain still developed after a brief delay, but it was substantially easier to tolerate.

Recovery Days 1 to 5. Fatigue and increased shoulder pain persisted briefly after the final injection, then improved. By the third recovery day my energy was about 75 percent of baseline and I could do yard work. By day 5 my sleep, energy, headache, stomach discomfort, and shoulder pain had largely returned to baseline.

Recovery Days 10 to 25. There was little to report by day 10. I later restarted KPV, GHK Cu, BPC 157, and TB 500. By day 20, brown spots on the backs of my hands appeared lighter. The improvement continued through day 25, but I saw no obvious hair change, reduction in nocturia, or lasting improvement in my shoulder.

Days 30 to 45. The skin changes became the main observable outcome. Brown age spots on both hands were markedly lighter. A longstanding lesion between two fingers changed from dark brown and slightly raised to lighter, nearly flat, and smoother. A second raised arm lesion that had returned after prior cryotherapy also became much flatter. The pigmentation on my hands and scalp continued to lighten through day 45.

Laboratory Results

Blood tests obtained on day 36 showed normal complete blood count and comprehensive metabolic panel results, with favorable lipids.

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Cardiovascular and metabolic. My LDL is 88, HDL 54, triglycerides 62 and ApoB 76 mg/dL; Lp(a) is 27 nmol/L. Glucose 90 mg/dL, A1c 5.3% and insulin 3.7 uIU/mL are reassuring for current glucose control.

Kidney and electrolytes. Creatinine 0.82 mg/dL and eGFR 94 mL/min/1.73 m^2 indicate preserved filtration. Sodium 137, potassium 4.1 mmol/L and calcium 9.0 mg/dL are in range. Urine protein and blood are negative.

Liver and pancreas. AST 20, ALT 18, ALP 47, GGT 7, amylase 47 and lipase 46 U/L are in range; bilirubin is 0.6 mg/dL.

Thyroid and hormones. TSH 3.48 mIU/L, free T4 1.0 ng/dL and free T3 2.7 pg/mL are in range. Total testosterone 567 ng/dL, free testosterone 68.0 pg/mL and LH 6.8 mIU/mL are in range.

Immune cells and nutrition. WBC 4.9 Thousand/uL, differential counts and platelets 211 Thousand/uL are in range. ANA is negative and RF <10. Vitamin D 58 ng/mL, MMA 207 nmol/L and homocysteine 8.4 umol/L are reassuring.

Interleukin 6. Within the laboratory reference range at 3.13 pg/mL. Because I lacked comparable pre-treatment measurements and several other therapies could affect these markers, the laboratory results do not establish a FOXO4-DRI effect.

Biological Interpretation

Mechanistic Extrapolation of FOXO4-DRI Induced Skin Changes: A plausible sequence is selective loss of susceptible senescent skin cells, clearance of cellular debris, reduced senescence-associated secretory signaling, lower melanogenic stimulation, and gradual epidermal and extracellular-matrix remodeling. That sequence explains why pigmentation and lesion texture continued to change for weeks after the brief exposure ended.

The observations support a narrow conclusion: this three-dose course produced biologically meaningful activity in skin. They do not determine whether 28 mg was optimal, whether three doses were necessary, if more courses would have had more effect, how much senolysis occurred, or whether comparable effects occurred elsewhere. Reduced inflammatory signaling, interruption of secondary senescence, vascular effects, and remodeling in other tissues remain plausible but unmeasured.

Because of these results, the several speculative or probable human benefits become more likely.

- **Reduced cutaneous senescent-cell burden and altered SASP signaling**
- **Reduced senescence-driven melanogenic signaling**
- **Ongoing skin remodeling**
- **Reduced local inflammatory/SASP signaling**
- **Senolysis in other accessible FOXO4-dependent tissues**

One particularly plausible additional target is senescent vascular endothelium.

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Bottom Line

The injections themselves were acutely uncomfortable but did not produce a recognized serious adverse reaction during the period covered by this diary. The visible skin changes were substantial and continued for more than a month. They are the strongest personal signal of biological activity, but causality is only inferred and systemic benefit remain uncertain.

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